NCE EXAM PREP

Practice Exam 27

Domain 3

Treatment Planning & Continuity of Care

26 Practice Questions | 34 minutes

A, B, C answer choices

Answer Key and Explanations included

> **Instructions.** Select the single best answer for each question. Record your answers separately. This domain practice set is drawn from Full-Length Practice Exam 05. Use the suggested 34-minute practice timer. Do not consult Part B until you complete Part A. These domain timings are editorial practice allocations, not official NBCC section limits.

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PART A — QUESTIONS

Choose one answer (A, B, or C) per question.

Question 1

> A client seeking career counseling has interest-inventory results spanning several fields. Interviews show that autonomy, predictable hours, and opportunities to learn are more important to the client than a particular occupation. The client wants to compare realistic options rather than receive a single career recommendation.

How should the counselor use the assessment results in the plan?

A) Develop exploratory interviews and option-comparison tasks organized around the client's stated values

B) Select the occupation with the highest inventory score and make application the first objective

C) Repeat the inventory until one occupational category clearly exceeds all others

Question 2

How does a treatment barrier differ from a contraindication?

A) A barrier always requires ending the goal, whereas a contraindication can be ignored with client consent

B) A barrier may call for added support or adaptation, whereas a contraindication may make a specific approach unsafe or unsuitable

C) A barrier concerns motivation, whereas a contraindication means the client is resistant to treatment

Question 3

What is the main purpose of a planned follow-up contact after discharge?

A) To check maintenance and access needs and determine whether additional support or reentry is indicated

B) To continue routine counseling informally without reopening or revising the treatment relationship

C) To verify that the client follows every recommendation exactly as the counselor originally wrote it

Question 4

> A client seeks counseling after a difficult divorce and has sleep and concentration problems that do not meet criteria for a mental disorder. The client wants help restoring routines and functioning and understands that counseling can address the concern without assigning a diagnosis.

Which initial plan is most appropriate?

A) Develop functional goals based on the presenting problems and client priorities without inventing a diagnosis

B) Delay all treatment planning until symptoms become severe enough to meet diagnostic criteria

C) Select the closest depressive diagnosis so that measurable objectives can be written

Question 5

A client with episodic migraine wants to resume enjoyable activities but cannot predict which days will involve a severe flare. The client can usually complete two brief activities during any seven-day period and wants an objective that remains measurable without requiring daily performance.

Which objective is most appropriate?

A) The client will complete one enjoyable activity every day regardless of migraine severity for six weeks

B) The client will participate more often whenever symptoms permit and discuss the experience later

C) The client will complete two chosen brief activities in each seven-day period for six weeks and record completion

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Question 6

A client in a multigenerational household says that contributing money and time to extended family is a central personal value. These commitments limit the client's availability for a career-training program, but the client does not view family obligations as a problem to eliminate.

Which planning response is most culturally and contextually responsive?

A) Make financial independence from relatives a preliminary goal before career planning begins

B) Explore training schedules and objectives that honor the client's stated family commitments and career interests

C) Exclude family obligations from the plan because they are not mental health symptoms

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Question 7

An integrated care provider referred a client for weight-related behavior change. The client rejects weight loss as the primary outcome but wants enough stamina to play with grandchildren and is willing to plan regular movement that does not worsen joint pain.

Which treatment goal is most collaborative?

A) Reach the weight target listed in the referral before addressing family activities

B) Increase exercise intensity until the client meets standard fitness recommendations

C) Improve participation with grandchildren through client-selected, medically appropriate movement objectives

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Question 8

A client with a terminal illness is entering home hospice and wants supportive counseling to continue. The current agency cannot provide visits in the home, but the hospice employs a counselor, and the client authorizes coordination.

Which plan best supports the client's transition?

A) Arrange a client-directed handoff to the hospice counselor, clarify current and receiving roles, and plan the ending

B) Continue office appointments because transferring counseling during terminal illness would be disruptive

C) End counseling without referral because hospice automatically addresses every emotional need

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Question 9

A discharge plan includes one follow-up telephone call one week after services end. The client does not answer at the agreed time. There was no acute safety concern at discharge, and the client authorized a neutral voicemail but no contact with family members.

What should the counselor do?

A) Contact a family member because failure to answer a follow-up call indicates probable crisis

B) Follow the agreed outreach procedure, leave the authorized neutral message, and document the attempt

C) Reopen the case immediately because a missed follow-up means discharge was premature

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Question 10

A client has met the agreed counseling goals and wants to terminate. A family member believes the client should continue and asks the counselor to postpone discharge. The client has decision-making capacity, no acute safety concern, and permits the counselor to hear the family member's concern but not to share clinical details.

What is the most appropriate response?

A) Continue treatment because family concern overrides termination criteria when relapse is possible

B) Refuse to hear the family member because no clinical details may be disclosed

C) Consider the concern within the client's permission, review aftercare with the client, and proceed collaboratively with termination

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Question 11

> An adult client wants family counseling with two adult children who live in different regions. All participants want to improve communication, can use private video connections, and are eligible for the agency's family telehealth service.

Which modality plan is most appropriate?

A) Require in-person attendance because family counseling cannot be effective through video

B) Begin individual counseling with the parent because geographically separated adults cannot share a family goal

C) Use the suitable family telehealth format and define shared goals, participation expectations, and review criteria

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Question 12

> Two partners began counseling with a shared goal of repairing their relationship. They have now decided to separate and both want help developing respectful co-parenting communication. Neither wants continued reconciliation work.

How should the treatment plan change?

A) Close the reconciliation goal and collaborate on a new co-parenting goal if it fits the service and both partners consent

B) Keep reconciliation as the primary goal because changing it may reinforce avoidance of relationship problems

C) End all counseling because separation makes any couples-based planning inappropriate

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Question 13

> A client with anxiety asks the counselor to create a detailed plan for repaying several high-interest debts. The counselor lacks financial-planning expertise. The client wants both emotional support and reliable guidance about repayment options.

Which plan best addresses these needs?

A) Refer the client to a qualified nonprofit financial counselor while continuing anxiety treatment and coordinating with permission

B) Design a repayment schedule because financial stress is contributing to the client's anxiety symptoms

C) Restrict counseling to emotion regulation and decline to discuss any outside resource for the debt concern

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Question 14

A client newly diagnosed with attention-deficit/hyperactivity disorder says, “This diagnosis means medication is the only real treatment.” The client wants organizational help but is uncertain about medication and has not yet developed the initial counseling plan.

What is the best way to review the diagnosis for planning purposes?

A) Avoid discussing treatment options until the client decides whether to take medication

B) Tell the client that medication decisions must be made before counseling goals can be written

C) Clarify what the diagnosis describes and collaboratively review counseling, support, and referral options without prescribing one path

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Question 15

What is the primary purpose of sequencing treatment objectives?

A) To ensure that every client completes the same foundational activities before individualized work begins

B) To address prerequisites and arrange achievable steps that build toward broader goals

C) To place the easiest tasks first regardless of their connection to the presenting problem

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Question 16

A cyberattack disables an agency's scheduling and record systems. The continuity protocol provides a verified offline contact roster, approved secure telephone procedures, and instructions not to use personal email or unapproved messaging. Several appointments are scheduled for the next two days.

Which action best follows the continuity protocol?

A) Use personal email temporarily because rapid client notification is more important than the approved procedure

B) Use the verified offline roster and approved telephone process to address appointments and urgent needs

C) Delay all contact until the electronic record is restored so no information is handled outside the system

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Question 17

A client with depression also has a diagnosed eating disorder characterized by compulsive exercise. The specialty medical team has instructed the client to avoid unsupervised exercise temporarily. A draft depression plan lists daily gym workouts as behavioral activation.

How should the counselor revise the plan?

A) Replace exercise-based activation with medically appropriate nonexercise activities and coordinate with the specialty team

B) Keep the gym objective because behavioral activation addresses depression rather than eating-disorder symptoms

C) Remove all behavioral activation because the temporary restriction rules out activity-based depression treatment

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Question 18

A counselor refers a client to a community employment program, but the program determines that the client does not meet its eligibility criteria. The client still wants vocational support, and two other programs serve people with the client's qualifications.

What should the counselor do next?

A) Close the vocational goal because the specialist program made an eligibility decision

B) Ask the client to repeat the original application without changing any information

C) Review the denial with the client, verify alternatives, and revise the referral plan

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Question 19

A client completing treatment for excoriation disorder identifies television viewing as a common setting for automatic skin picking. The client has successfully used a competing response and keeps a textured object nearby during sessions at home.

Which maintenance plan best supports relapse prevention?

A) Avoid television permanently because exposure to a familiar cue makes recurrence unavoidable

B) Continue the competing response and stimulus supports, monitor early recurrence, and define when to seek follow-up

C) Remove the textured object to determine whether the urge has been fully eliminated before discharge

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Question 20

A client has attended counseling reliably and used the agreed interventions, but disabling health anxiety remains unchanged after two well-documented plan revisions. The client wants a clinician with specialized expertise, and an appropriate program is available.

Which next step best supports continuity?

A) Collaboratively arrange the specialty referral and provide an agreed bridge while the new service evaluates fit

B) Repeat the current plan for another full cycle because referral could undermine the client's confidence

C) End the case before making the referral so the specialist receives an unbiased presentation

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Question 21

Which feature most clearly makes an objective relevant?

A) It can be completed quickly and produces data that are easy for the counselor to graph

B) It advances the client's identified goal and addresses information supported by the case formulation

C) It appears in a standard protocol commonly used for the client's diagnosis

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Question 22

A client consistently completes detailed daily thought records for depression but reports spending up to two hours analyzing each entry and feeling more stuck afterward. The records show little connection between additional detail and mood improvement.

How should the counselor use this outcome information?

A) Increase the required detail because more complete records may eventually reduce rumination

B) Replace the burdensome task with a briefer strategy and monitor whether mood and functioning improve

C) Keep the intervention unchanged until the original treatment period ends so the trial remains standardized

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Question 23

A client says the current counselor's structured style is not a good fit and requests transfer to another clinician in the agency. The client has no acute safety concern, wants treatment to continue, and agrees to a transition discussion.

Which response best protects continuity?

A) Persuade the client to remain for several more months so the relationship has time to improve

B) End the case immediately so the client's preference does not influence the receiving clinician

C) Explore transition needs, facilitate the chosen transfer, and clarify information sharing and appointment timing

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Question 24

An observant Orthodox Jewish client is interested in a stress-management group but cannot attend its Saturday meetings. An equivalent weekday group has openings and meets the same clinical need.

Which planning decision is most appropriate?

A) Recommend the Saturday group because treatment schedules should remain separate from religious practice

B) Ask the client to try one Saturday session before determining whether attendance is a genuine barrier

C) Offer the equivalent weekday group and document the client's selected feasible modality

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Question 25

A client with recurrent depression moved from weekly counseling to monthly maintenance after sustained improvement. The maintenance plan states that missing work twice in one week and withdrawing from a weekly community activity will trigger review for increased support. Both events occur this week, and the client requests help early.

What is the best continuity response?

A) Use the predefined criteria to review needs and temporarily increase support before further deterioration

B) Maintain monthly visits until the client again meets full diagnostic criteria for a depressive episode

C) Discharge the client because needing more support shows that maintenance counseling was ineffective

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Question 26

> A 4-year-old in preschool hits peers most often when play ends without warning. The child responds to picture cues, can request “one more minute,” and enjoys praise from the teacher. The counselor, caregiver, and teacher are developing a consistent plan.

Which behavioral plan best fits the available information?

A) Use time-out after every hit without changing transition cues or teaching another response

B) Allow each adult to handle transitions differently so the child learns to adapt across settings

C) Provide a visual countdown, teach the request, reinforce safe transitions, and record hitting frequency

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PART B — ANSWER KEY AND EXPLANATIONS

Answers match the numbered items in Part A.

Question 1 — Correct: A

Correct Answer: Develop exploratory interviews and option-comparison tasks organized around the client's stated values

Explanation: The broad interest pattern is useful when combined with the client's priorities and real-world exploration. Treating the highest score as a directive or repeating the inventory until it produces one answer would overstate what the assessment can decide.

Question 2 — Correct: B

Correct Answer: A barrier may call for added support or adaptation, whereas a contraindication may make a specific approach unsafe or unsuitable

Explanation: A barrier can often be reduced through access changes, supports, pacing, or sequencing while the goal remains appropriate. A contraindication concerns the suitability or safety of a particular approach under the current conditions, and neither concept simply labels client resistance.

Question 3 — Correct: A

Correct Answer: To check maintenance and access needs and determine whether additional support or reentry is indicated

Explanation: A planned follow-up evaluates how the client is functioning after discharge and whether the aftercare plan remains sufficient. It does not silently continue treatment or function as surveillance of perfect compliance.

Question 4 — Correct: A

Correct Answer: Develop functional goals based on the presenting problems and client priorities without inventing a diagnosis

Explanation: Counseling goals can address real distress and impairment even when no mental disorder is established. Delaying care or assigning an unsupported diagnosis would misrepresent the available information.

Question 5 — Correct: C

Correct Answer: The client will complete two chosen brief activities in each seven-day period for six weeks and record completion

Explanation: The objective remains specific, measurable, and time-limited while accommodating an unpredictable episodic condition. A rigid daily requirement may be unattainable during flares, whereas “more often” lacks a defined target.

Question 6 — Correct: B

Correct Answer: Explore training schedules and objectives that honor the client's stated family commitments and career interests

Explanation: The client defines family contribution as a value rather than a symptom or obstacle to remove. Collaborative planning can seek feasible training paths that integrate both commitments without imposing an individualistic definition of independence.

Question 7 — Correct: C

Correct Answer: Improve participation with grandchildren through client-selected, medically appropriate movement objectives

Explanation: The goal reflects the client's desired functioning and incorporates the joint-pain constraint without making weight the sole outcome. A referral target or standard exercise level should not replace the client's informed priorities.

Question 8 — Correct: A

Correct Answer: Arrange a client-directed handoff to the hospice counselor, clarify current and receiving roles, and plan the ending

Explanation: The hospice counselor can provide accessible ongoing support that the current agency cannot deliver at home. A coordinated, preference-sensitive handoff preserves continuity without assuming hospice meets every need automatically or requiring burdensome office visits.

Question 9 — Correct: B

Correct Answer: Follow the agreed outreach procedure, leave the authorized neutral message, and document the attempt

Explanation: The counselor should implement the specific follow-up permission and plan established at discharge. A missed call without additional risk information does not authorize family contact or automatically reopen treatment.

Question 10 — Correct: C

Correct Answer: Consider the concern within the client's permission, review aftercare with the client, and proceed collaboratively with termination

Explanation: The capable client has met the criteria and controls both termination and the permitted scope of family involvement. The concern may inform aftercare discussion but does not automatically override the client's decision.

Question 11 — Correct: C

Correct Answer: Use the suitable family telehealth format and define shared goals, participation expectations, and review criteria

Explanation: The facts establish willingness, privacy, eligibility, and a shared family concern, making telehealth a feasible modality. Geography alone does not require individual treatment or make family work ineffective.

Question 12 — Correct: A

Correct Answer: Close the reconciliation goal and collaborate on a new co-parenting goal if it fits the service and both partners consent

Explanation: The original goal no longer reflects either partner's preference, while a shared co-parenting need remains. Updating the plan preserves collaboration and service relevance rather than treating separation as failure or a reason to retain reconciliation work.

Question 13 — Correct: A

Correct Answer: Refer the client to a qualified nonprofit financial counselor while continuing anxiety treatment and coordinating with permission

Explanation: The client needs expertise beyond the counselor's competence as well as ongoing help with the emotional effects of debt. Parallel referral and counseling address both needs without the counselor practicing financial planning or ignoring a major stressor.

Question 14 — Correct: C

Correct Answer: Clarify what the diagnosis describes and collaboratively review counseling, support, and referral options without prescribing one path

Explanation: An ADHD diagnosis can inform planning without dictating medication or making medication a prerequisite for counseling goals. Clear review corrects the client's misconception and supports informed participation in available options.

Question 15 — Correct: B

Correct Answer: To address prerequisites and arrange achievable steps that build toward broader goals

Explanation: Sequencing creates a clinically meaningful progression when one skill, resource, or step supports the next. It is not a requirement for identical preparation or for choosing unrelated tasks merely because they are easy.

Question 16 — Correct: B

Correct Answer: Use the verified offline roster and approved telephone process to address appointments and urgent needs

Explanation: The continuity protocol supplies a secure alternative that permits timely contact while electronic systems are unavailable. Personal accounts would bypass the stated safeguards, whereas waiting could leave scheduled or urgent needs unattended.

Question 17 — Correct: A

Correct Answer: Replace exercise-based activation with medically appropriate nonexercise activities and coordinate with the specialty team

Explanation: The exercise restriction makes the drafted intervention unsuitable, not behavioral activation as a whole. Alternative rewarding activities can support depression care while respecting the specialty team's medical guidance.

Question 18 — Correct: C

Correct Answer: Review the denial with the client, verify alternatives, and revise the referral plan

Explanation: Ineligibility for one program does not eliminate the vocational goal when other appropriate resources exist. A responsive continuity plan uses the feedback to select and verify a better-matched option.

Question 19 — Correct: B

Correct Answer: Continue the competing response and stimulus supports, monitor early recurrence, and define when to seek follow-up

Explanation: The plan extends strategies that have worked in the client's identified high-risk setting and adds early-response criteria. Permanent avoidance is unnecessary, while removing supports to test recovery would discard protective gains.

Question 20 — Correct: A

Correct Answer: Collaboratively arrange the specialty referral and provide an agreed bridge while the new service evaluates fit

Explanation: Reliable participation and two documented revisions have not produced improvement, and the client requests suitable expertise that is available. A coordinated referral with a bridge avoids both unproductive repetition and a gap before acceptance.

Question 21 — Correct: B

Correct Answer: It advances the client's identified goal and addresses information supported by the case formulation

Explanation: Relevance requires a meaningful link between the objective, the client's desired outcome, and the conceptualized problem. Speed, graphability, or protocol inclusion alone does not supply that connection.

Question 22 — Correct: B

Correct Answer: Replace the burdensome task with a briefer strategy and monitor whether mood and functioning improve

Explanation: The client completes the assignment, but the available outcome information links excessive detail with rumination and no demonstrated benefit. Revising the intervention tests a better fit rather than equating more effort with greater effectiveness.

Question 23 — Correct: C

Correct Answer: Explore transition needs, facilitate the chosen transfer, and clarify information sharing and appointment timing

Explanation: The client has identified a fit concern and wants care to continue, making a planned transfer appropriate. Pressuring the client to remain or ending without coordination would create avoidable barriers to ongoing treatment.

Question 24 — Correct: C

Correct Answer: Offer the equivalent weekday group and document the client's selected feasible modality

Explanation: The weekday group meets the same clinical need while respecting a clearly stated scheduling constraint. Requiring a violation of observance or a trial of the inaccessible time would create an unnecessary barrier.

Question 25 — Correct: A

Correct Answer: Use the predefined criteria to review needs and temporarily increase support before further deterioration

Explanation: The maintenance plan identifies these functional changes as early signals for review, and the client is seeking help at that point. Waiting for a full episode would defeat the purpose of stepped maintenance, while discharge would misinterpret appropriate early reengagement as failure.

Question 26 — Correct: C

Correct Answer: Provide a visual countdown, teach the request, reinforce safe transitions, and record hitting frequency

Explanation: The plan addresses the predictable antecedent, teaches an available replacement response, uses an effective reinforcer, and creates outcome data. Punishment alone or inconsistent adult responses would not use the functional information supplied.
